



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

YouthReach South Duress Response

Reference #:FSFH-MENH-POL-0025

Scope

Site	Service/Department/Unit	Disciplines
YouthReach South	Cockburn Youth Centre	Nursing, Medical, Allied Health

1. Introduction

YouthReach South (YRS) is a specialist youth mental health service providing counselling, therapy and care coordination to young people aged 13 – 24 years in the Metropolitan Health Service catchment. YRS endeavors to meet its Workplace Health and Safety responsibilities to protect staff from harm by providing duress alarms wherever they have been deemed necessary. This document outlines the procedure for responding to a duress alarm when activated to maximise the safety of clients, carers, families and staff.

2. Terminology

Fixed duress alarm	A hard-wired alarm system with duress buttons strategically placed throughout the building.
Personal duress alarm	Portable alarms accessible through a downloaded smart phone application for use in the community.

3. Policy

Staff members are not to engage an aggressive person where the level of violence exceeds their capacity to manage the risk.

Staff members must not attempt restraint in the community setting under any circumstances.

The information provided in this document is based on its relevance to Fiona Stanley Fremantle Hospital Group (FSFHG)

only. Due to differences in context, scope of practice and differences in service delivery, FSFHG does not make claim to its relevance or appropriateness for use within other organisations/sites.

- Orientation to duress alarms and the YRS duress response procedure will be included in new staff induction processes.
- YRS Team Leader will ensure that all staff members participate in duress response drills annually.
- Staff are to activate a duress alarm in a situation where they feel that their personal safety or the safety of another is threatened.

3.1. Risk Assessment of New/Unknown Clients

- All YRS clients are voluntary.
- A risk assessment will be undertaken via review of all available historical and current information (including PSOLIS alerts) prior to the initial assessment interview.
- Whenever possible initial assessment interviews are to be carried out by two staff members.
- Where a potential risk of aggression is identified, a safety plan must be established in consultation with the multidisciplinary team (MDT).
- Where there is a risk of aggression and it is assessed that the risk may not be contained on the premises at YRS, an alternative site is to be identified or phone/telehealth considered.

4. On site procedure

4.1. General considerations

- YRS is located on the first floor of the Cockburn Youth Centre building. The following risk management strategies are in place including:
- Fixed duress alarms are located on the wall of each of the two YRS interview rooms (rooms 4 and 5) and in the reception area.
- Clients are unable to access the YRS area of the building without security swipe.
- If a client is already in the building, they are unable to access staff areas without security swipe.

4.2. Maintenance and Testing

- Fixed duress alarms and panel will be serviced every 6 months by a licensed technician and a record of this documented in the YRS Evidence Folder.
- Fixed duress alarms are to be checked quarterly by staff and a record of this is to be documented in the YRS Evidence Folder.
- Any faulty duress alarm is to be reported to the Team Leader and WHS representative immediately. Staff will be informed, and arrangements made for immediate repair. The interview room will be deemed unsuitable for clinical use until the duress alarm is operational.
- If there are no other interview rooms available, alternative options such as telehealth or seeing the client at another community mental health clinic should be considered for service delivery.

4.3. Client Interview/Appointment

- Administration staff must always be informed when a client interview is taking place.
- Clinicians external to YRS are not to see clients with elevated risk at YRS without prior approval of YRS Team Leader or delegate.
- Where possible there should be a minimum of two staff onsite whilst a client interview takes place (three staff with a new client). One of the staff members can be administrative staff.
- All clients are escorted to the interview room by a member of staff/clinician.
- Clinician/s will position themselves in the interview room, where they have unrestricted access to the duress alarm and to egress the interview room (where a client is well known to the clinician/s and the likelihood of aggression is minimal, the clinician can use clinical judgement to determine seating arrangements)

4.4. Duress activation

- The staff member, who has activated the duress, may withdraw from the situation if appropriate.
- A blue light on the corridor ceiling outside of the interview room and reception will notify staff of the location of the duress when activated.
- When the alarm sounds it is audible in all YRS areas of the building.
- The duress alarm is monitored by Satellite Security and when activated they will contact YRS reception to enquire what type of emergency assistance is required.
- If YRS reception does not answer the call, Satellite Security will contact CoSafe Security and request they attend.
- All staff on the premises will proceed quickly and calmly to the area outside of the interview room/reception and first on the scene will look through the peephole/door viewer and communicate the situation.
- If assistance is required, administration staff/clinician will convey appropriate information to Cockburn Youth Centre Staff via telephone.

4.5. Duress response

- The most senior clinical staff member present will assess the situation and provide the leadership role. Where senior clinical staff members are at equal levels, the first senior clinician to attend will take on the leadership role.
- The leader will direct interventions to contain or prevent escalation of the situation. They may also delegate staff to:
- Liaise with emergency personnel (Police, Ambulance etc.) as required.
- Clear the waiting area and direct all persons to a safe space, as required.
- Intervene, where indicated, without physical intervention, or direct staff to enter the area where the incident is occurring, using de-escalation and breakaway techniques as required.
- Appropriately trained staff are to Administer basic first aid where indicated and safe to do so.

- Coordinate communication with other staff in the building and those likely to return/enter building.
- Reception staff will cancel the duress alarm as directed by the leader / emergency services.

5. Community visiting procedure

5.1. General considerations

- Where community visits are undertaken, clinicians will follow the SMHS Community/ Home Visiting Policy.
- Staff working with or visiting clients in the community will have access to the emergency response application Emergency Plus App on their smart phone.

5.2. Maintenance and testing

- Prior to a community visit staff will check the Emergency Plus App.
- Staff will update the App as prompted by the software system.

5.3. Duress activation

- Staff members will carry a SMHS issued Smart phone or be accompanied by another staff member who has one on their person when working with or visiting clients in the community.
- Staff will be instructed on the use of the community-based duress system prior to use in the community.
- A duress in the community can be activated by calling 000 on the work provided smart phone by:
 - Opening the Emergency Plus App
 - Tapping on the relevant icon e.g. 000 Emergency which will connect the staff member to a Triple zero call taker
 - When asked for location read out street location where available or the 3 words displayed at the bottom of the screen in the 'My location' section
 - Help will be sent to the exact location.

6. Post Incident

- The Team Leader should be informed (if not already aware).
- Where indicated, the Team Leader will ensure that clinical incident reporting is completed in line with Clinical Incident Management Policy (i.e. if there is harm to a client).
- Where indicated, the Team Leader will ensure that appropriate WH&S reporting is completed (i.e. if injury to staff or staff risk issue/hazard identified) using the Combined Hazard or Incident Reporting (CHoIR) system.
- The event should be recorded within the client's medical record (and alert entered on PSOLIS).
- Team leader to inform Service Director or delegate, of the incident.
- Formal debriefing to be arranged as required by the Team Leader.

- Incident to be discussed with MDT as soon as possible and plan for future care to be developed.

7. Compliance Monitoring

- The YRS Team Leader will monitor compliance with:
 - number of staff trained in API.
 - percentage of staff participating in annual duress response drills.

8. Related Policy Documents

- WA Health:
 - [Clinical Incident Management Policy SMHS:](#)
 - [Community/Home Visiting SMHS:131](#)
 - [Workplace Aggression & Violence SMHS:127](#)
 - [Critical Incident Stress Management](#)
- FSFHG
 - Clinical Risk assessment and Management FSFH-HW-POL-0017
 - FH Duress Alarms and Response FSFH-MENH-PRO-0005

9. Related Standards

- NSQHS Standards:
 - Clinical Governance
- Chief Psychiatrists Standards for Clinical Care:
 - Risk Assessment and Management

10. Authorisation

EXECUTIVE SPONSOR: S5 Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	07/2022	Team Leader YRS	S5 SQR Committee	S5 SQR Committee	09/2026
2	10/2022	Team Leader YRS	S5 SQR Committee	S5 SQR Committee	10/2026
3	02/2024	Team Leader YRS	Manager, Emergency Mx	S5 SQR Committee	05/2028